Meridian Health Connect — Complete Program Scope for Vendor Proposal

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MERIDIAN CARE NETWORK, INC. — REQUEST FOR PROPOSAL

Meridian Health Connect — Complete Program Scope

Vendor proposal attachment for architecture, phased implementation, pilot, deployment, and support

Field	Detail
Document ID	MC-RFP-SCOPE-0001
Version	1.0
Status	PUBLIC RFP ATTACHMENT — SANITIZED
Prepared	2026-08-11
Issuing organization	Meridian Care Network, Inc.
Product	Meridian Health Connect
Response type	Overall phased program with separate phase timelines and staffing plans

Purpose: Obtain comparable, independent proposals from qualified vendors on scope, architecture, and delivery approach.

Commercial status: This is a scope and capability solicitation only. No development authorization, budget figure, or funding commitment is implied or attached to this document. Bidders should direct any commercial or contracting discussion to the process described in Section 15; this attachment contains no pricing information and vendors should not include pricing in their response to this attachment.

Scope notice: This attachment describes the complete intended Meridian Health Connect program at a proposal level. It is not a final software requirements specification, statement of work, contract, or authorization to begin development. Detailed clinical rule packs, payer-connectivity credentials, security control specifics, legal work product, and proprietary operating materials will be provided only to shortlisted firms under controlled access.

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1. Proposal objective and required response structure

Meridian Care Network, Inc. ("Meridian") is requesting proposals from experienced healthcare software architecture and development organizations for the design and phased construction of Meridian Health Connect, a multi-tenant care-coordination and patient-engagement operating platform intended for use by independent clinics, multi-site practice groups, and affiliated care-management organizations across the United States.

The selected organization must be capable of producing architecture, backend services, clinical workflow engines, integrations with payer and laboratory systems, web and mobile experiences, testing, deployment, documentation, and ongoing support.

The response is not one undivided narrative: return one overall program plan, but break it into controlled phases. Architecture/foundation and Pilot Zero must be described separately from later clinical-domain increments. Later phases may be described at a planning level of detail until their detailed requirements are issued to shortlisted firms.

1.1 Required response classes

- A defined scope and staffing plan for architecture confirmation, technical discovery, final Pilot Zero planning, and the required architecture-response artifacts.
- A defined scope, assumptions, exclusions, dependencies, and acceptance evidence plan for Pilot Zero.
- Separate phase descriptions for ambulatory scheduling, chronic-care management, remote patient monitoring, behavioral health, payer connectivity, and other later capabilities.
- An overall program schedule labeled by confidence level, accompanied by a phase-by-phase timeline.
- A description of what would need to be resolved before any later phase could be scoped with confidence.

1.2 Schedule response

Provide both a responsible standard schedule and any accelerated option. State specifically whether architecture plus Pilot Zero can be completed within five months or sooner after written authorization without weakening tenant isolation, clinical safety, audit, security, legal controls, testing, backup/restore, or rollback. Do not interpret this as a five-month expectation for the complete national product.

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2. Business and product overview

Meridian Health Connect is intended to become a complete care-coordination and patient-engagement platform for independent clinics, multi-site practice groups, remote-monitoring programs, behavioral health providers, and affiliated care-management organizations. It is not merely a scheduling calendar or a messaging add-on, and it is not a collection of disconnected point tools.

The platform is expected to combine scheduling, secure messaging, care plans, remote monitoring data, referral coordination, documentation support, payer eligibility and claims-adjacent workflows, patient portal access, and human-supervised clinical decision support in one shared but strongly isolated core. Clinics must receive usable operational and clinical intelligence rather than static reports alone.

2.1 Intended market and deployment model

- Commercial multi-tenant SaaS offered to outpatient clinics and affiliated care-management organizations throughout the United States.
- Clinic tenants may be single-location, multi-location, single-entity, or approved multi-entity organizations.
- Affiliated care-management organizations and remote-monitoring programs require protected, independently governed tenant boundaries.
- Managed cloud deployment with production, staging, test, and development separation; no clinic self-hosting dependency.
- Responsive web experiences plus mobile applications or mobile-optimized workflows for care teams and patients.
- API-first and event-capable architecture supporting replaceable outside providers (labs, payers, pharmacy networks) and Meridian-owned data portability.

2.2 Business outcome

Mission: build the most complete, trustworthy, usable, and intelligence-driven care-coordination platform available to independent and affiliated outpatient providers, while remaining practical for daily clinical work and safe for regulated health information and clinical decision support.

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3. Non-negotiable architecture principles

Principle	Required meaning
One governed core	Meridian Health Connect, the affiliated care-management profile, and future remote-monitoring capabilities are profiles, configurations, entitlements, workflows, or bounded services on a shared architecture — not uncontrolled software forks.
Backend before screen volume	Canonical records, rules, events, permissions, care-plan logic, audit, reconciliation, and failure behavior are designed before broad dashboard or page construction.
Tenant isolation	Tenant is the primary protected data and authorization boundary. Cross-tenant access is exceptional, explicit, least-privileged, visible, revocable, and audited.
Immutable clinical history	Originated encounters, care-plan changes, documentation, audit records, and historical states are corrected through amendment, addendum, or tracked change — never silent overwrite.
Deterministic	Scheduling, eligibility verification, appointment-reminder timing, and

Principle	Required meaning
scheduling and eligibility	referral state must be reproducible and testable.
Safe failure	A missing payer rule, unconfirmed lab result, ambiguous eligibility state, or unreconciled referral outcome must stop, queue, or degrade safely rather than guess.
Human authority	AI may recommend, draft, and summarize, but cannot autonomously finalize a diagnosis, alter a care plan, approve a referral, transmit a prescription, or make other consequential clinical decisions.
Pilot before market	Every material capability must pass controlled pilot and evidence-based acceptance before general commercial release.
Patient data rights	Patient-authorized export, portability, retention, deletion treatment (subject to legal retention requirements), and access-request procedures must be designed from the start.
Traceable change	Requirements, decisions, risks, questions, tests, code, releases, and evidence require stable identifiers and controlled version history.

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4. Operating models, tenancy, and organizational boundaries

Operating situation	Required boundary
Independent clinic	Separate tenant by default; tenant controls its own patients, schedules, encounters, documentation, users, integrations, and reporting.
Same-owner multi-site group	May use one tenant with locations or separate tenants, subject to legal-entity, payer-contract, billing, and reporting needs.
Multiple legal entities	If approved within one tenant, every entity must remain explicit across payer contracts, billing identifiers, permissions, and reports.
Affiliated care-management organization	Each separately contracted care-management affiliate is isolated as its own tenant even when common ownership exists; authorized corporate oversight must be contractual, permissioned, and audited.
Remote-monitoring program partner	Always a separate tenant with separate users, device data, documents, reporting, and controlled patient-transfer workflow.
Meridian support	Purpose-specific, time-limited, least-privileged, revocable, visible, and fully audited support access.
Cross-tenant clinician	Independent assignment and approval in each tenant; no combined search, patient history, or account access by default.
Standalone remote-monitoring customer	May purchase a bounded service such as device-data intake without purchasing the complete Meridian Health Connect platform.

4.1 Pilot model

- Two separately isolated outpatient-clinic pilot environments are planned for the initial foundation proof, located in different states to exercise state-specific rule differences.
- Patient, scheduling, documentation, and clinical data must not flow between those pilots.
- A specifically approved patient-transfer workflow (e.g., a patient relocating between affiliated clinics) may cross the boundary with immutable before/after evidence and authorized consent.
- Later affiliated care-management pilots will operate at additional sites and remain outside Pilot Zero unless added by approved change control.
- All later capabilities must be pilot-tested before general market release.

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5. Program map: ten clinical/operational domains and five shared foundations

ID	Domain	Primary responsibility
D01	Patient Access & Scheduling	Intake, eligibility checks, appointment scheduling, waitlists, reminders, and cancellation handling.
D02	Patient, Household & CRM	Tenant-owned patient records, roles, household relationships, communications, and retention.
D03	Clinical Documentation & Encounters	Encounter capture, structured notes, templates, amendments, and documentation workflow.
D04	Care Plans & Chronic Disease Management	Goal setting, care-team tasks, monitoring targets, and longitudinal plan tracking.
D05	Referral & Care Coordination	Referral creation, tracking, specialist communication, and closed-loop confirmation.
D06	Remote Patient Monitoring	Device onboarding, data intake, threshold alerts, and clinician review workflow.
D07	Payer Eligibility & Coverage	Eligibility verification, coverage details, authorization status, and payer rule handling.
D08	Behavioral Health Operations	Screening tools, safety-plan workflow, specialized documentation, and elevated privacy handling.
D09	Patient Communication & Portal	Secure messaging, portal access, reminders, forms, and patient-facing self-service.
D10	Practice Operations & Reporting	Staffing, task queues, quality-measure tracking, and operational reporting.

5.1 Shared enterprise foundations

ID	Foundation	Primary responsibility
F11	Platform Core, Identity & Workflow	Tenant, entity, location, identity, authorization, configuration, workflow, document, search, communication, and audit.

ID	Foundation	Primary responsibility
F12	Data, Analytics & Clinical Decision Support	Governed measures, explanations, predictions, recommendations, and privacy-safe benchmarking.
F13	Integration, APIs & Patient Data Rights	Adapters, API/event contracts, portability, monitoring, reconciliation, and failure recovery.
F14	Compliance, Privacy, Security & AI Governance	Effective-dated federal/state health-information controls, privacy, security, evidence, and AI authority.
F15	Experience, Reliability, Training & Customer Success	Usability, accessibility, observability, recoverability, support, onboarding, and future clinical-education modules.

6. Detailed functional scope by domain

6.1 D01 — Patient Access & Scheduling

- Multi-location, multi-provider scheduling with configurable visit types, durations, and resource requirements (rooms, equipment, telehealth links).
- Real-time or near-real-time payer eligibility checks at scheduling and again at check-in, with cached results and staleness indicators.
- Waitlist management, overbooking rules, recall scheduling, and automated reminder sequencing (SMS/email/voice) with configurable timing and opt-out handling.
- Self-scheduling via patient portal for approved visit types, subject to clinic-defined rules and blackout windows.
- No-show and late-cancellation tracking with configurable clinic policy application.
- Multi-provider, multi-resource conflict detection and double-booking prevention with explicit override and reason capture.

6.2 D02 — Patient, Household & CRM

- Tenant-owned patient records with governed roles: patient, guardian/proxy, caregiver, guarantor, and emergency contact.
- Identity evidence, contact points, addresses, insurance information, preferences, provenance, corrections, and privacy requests.
- Patient timeline covering scheduling, encounters, care plans, referrals, monitoring data, communications, and portal activity.
- Household and related-party relationships without creating cross-clinic visibility or a global patient history available to unrelated tenants.
- Role-based masking and field-level restrictions for sensitive identity, behavioral-health, substance-use, and insurance data, consistent with applicable federal confidentiality rules for specially protected records.

6.3 D03 — Clinical Documentation & Encounters

- Encounter creation tied to a scheduled visit or ad hoc entry, with structured and free-text documentation support.

- Configurable templates by visit type and specialty, with required-field enforcement and clinician override with reason.
- Amendment and addendum workflow preserving the original entry; no destructive edit of finalized clinical documentation.
- Co-signature workflow for supervised clinicians and trainees where applicable.
- Document/media attachment (images, scanned forms, external records) with provenance and viewer-access controls.
- AI-assisted draft summarization of encounter notes from structured inputs, subject to mandatory clinician review and explicit acceptance before the note is finalized.

6.4 D04 — Care Plans & Chronic Disease Management

- Longitudinal care-plan records with goals, target measures, interventions, owning care-team member, and review cadence.
- Task generation and assignment across care-team roles (clinician, care coordinator, medical assistant) with due dates and escalation.
- Configurable condition-specific templates (e.g., diabetes, hypertension, COPD) editable by clinic-level clinical staff without code changes.
- Patient-facing goal visibility and progress display within the portal, gated by clinic configuration.
- Care-plan history and versioning; changes require an actor, timestamp, and reason where the change affects an active goal or intervention.

6.5 D05 — Referral & Care Coordination

- Referral creation with reason, urgency, target specialty/provider, and supporting documentation attachment.
- Referral status tracking (sent, received, scheduled, completed, declined) with configurable follow-up tasks on stalled referrals.
- Closed-loop confirmation capturing specialist consult notes or an equivalent outcome record where available.
- Directory of external specialists and facilities with tenant-specific preferred-provider lists.
- Referral aging and exception dashboards for care coordinators.

6.6 D06 — Remote Patient Monitoring

- Device onboarding and assignment workflow (blood pressure cuffs, glucometers, weight scales, pulse oximeters, and similar approved categories).
- Structured intake of device-reported readings via approved integration partners, with a clear record of data source, timestamp, and device identity.
- Configurable threshold-based alerting with routing to the responsible clinician or care coordinator and required acknowledgment.
- Trend visualization and adherence tracking (reading frequency vs. expected cadence) at the patient and program level.
- The platform must not claim to interpret or diagnose from monitoring data; all clinical interpretation remains a human clinician action.

6.7 D07 — Payer Eligibility & Coverage

- Real-time or batch eligibility verification against connected payers, with coverage summary, copay/deductible information where available, and authorization requirement flags.
- Prior-authorization status tracking (submitted, pending, approved, denied) with supporting-document attachment.
- Coverage-change detection and re-verification triggers at defined intervals or on scheduling events.
- Clear differentiation between confirmed payer data and clinic-entered estimates, with provenance retained.

6.8 D08 — Behavioral Health Operations

- Configurable standardized screening-tool administration (e.g., depression and anxiety screeners) with scoring and threshold-based alerting to the care team.
- Safety-plan documentation workflow with elevated-urgency routing and required acknowledgment.
- Segregated documentation visibility consistent with applicable confidentiality requirements for behavioral-health and substance-use records, including consent-gated release.
- Specialized note templates distinct from general medical documentation, with independent access controls.

6.9 D09 — Patient Communication & Portal

- Secure, encrypted messaging between patients and care teams, with configurable response-time expectations and escalation for unanswered messages.
- Patient portal for appointment view/self-scheduling, secure messaging, form completion, care-plan visibility, and document access, gated by clinic and program configuration.
- Digital intake and consent-form workflow with structured field capture and signature evidence.
- Broadcast and targeted communication tools for appointment reminders, recalls, and program-specific outreach, with consent and opt-out enforcement.

6.10 D10 — Practice Operations & Reporting

- Staff task queues by role, urgency, and due date, with shift handoff support.
- Quality-measure tracking dashboards (e.g., screening completion rates, monitoring-program adherence, referral closure rates) without exposing one clinic's identifiable data to another.
- Operational reporting for scheduling utilization, no-show rates, documentation-completion timeliness, and care-coordinator workload.
- Exportable, role-gated reports for clinic leadership and affiliated-organization oversight where contractually authorized.

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7. Shared enterprise foundation scope

7.1 F11 — Platform Core, Identity & Workflow

- Tenant, legal entity, location, department, user, role, permission, policy, entitlement, feature flag, configuration, and environment foundation.
- Single sign-on options, multi-factor authentication, session/device controls, account recovery, privileged administration, support access, periodic access review, and segregation of duties.
- Workflow/state engine, tasking, approvals, reason codes, thresholds, queues, timers, notifications, documents, signatures, search, communications, and immutable audit.
- Stable internal identifiers, effective-dated configuration, record versioning, correction patterns, idempotent commands, events, jobs, and failure recovery.
- Clinic/group/affiliate onboarding, configuration, data migration, sandbox/test setup, subscription/entitlement, and controlled offboarding.

7.2 F12 — Data, Analytics & Clinical Decision Support

- Governed operational and quality measures with definitions, owners, source fields, calculation versions, freshness, lineage, access, and reconciliation.
- Role-based dashboards, drilldowns, scheduled reports, alerts, anomaly detection, and export.
- Tenant-specific intelligence for scheduling utilization, monitoring-program adherence, referral closure, and care-plan progress.
- Privacy-safe anonymous benchmarking only after contractual, privacy, legal, minimum-population, suppression, and governance controls are approved.
- Provider-agnostic AI gateway, approved models, prompt/version registry, grounding, mandatory human review, logging, evaluation, cost controls, monitoring, rollback, and patient-data-use choices.

7.3 F13 — Integration, APIs & Patient Data Rights

- Versioned API and event contracts, replaceable provider adapters, credential isolation, tenant account ownership, sandbox/live separation, retries, timeouts, dead-letter handling, observability, and reconciliation.
- Provider-result states that distinguish requested, accepted, pending, confirmed, failed, timed out, duplicated, reversed, and unknown outcomes.
- Patient-authorized export, ongoing extracts, documents, media, reference data, audit evidence, and migration/termination support in documented, standards-based formats.
- Developer documentation, webhooks, rate limiting, pagination, idempotency, authentication, authorization, versioning, monitoring, and deprecation policy.

7.4 F14 — Compliance, Privacy, Security & AI Governance

- Effective-dated jurisdiction packs for applicable federal and state health-information law, consent requirements, retention, minimum-necessary access, and prohibited behavior.

- Security architecture for encryption, key/secrets management, tokenization, sensitive-field isolation/masking, least privilege, secure SDLC, dependency management, vulnerability testing, and incident response.
- Privacy notice/consent, purpose limitation, access/copy/correction/deletion treatment, legal holds, retention, minimization, export, breach response, and vendor/data-processing controls appropriate to protected health information.
- Immutable audit evidence for viewing, creating, changing, approving, overriding, exporting, supporting, and accessing sensitive information.
- AI governance defining permitted, prohibited, and human-approved actions; model/vendor review; bias/fairness testing; output labeling; source evidence; monitoring; and incident/rollback procedures.

7.5 F15 — Experience, Reliability, Training & Customer Success

- Consistent role-based navigation, responsive design, accessibility, keyboard support, plain-language errors, progress/save behavior, mobile field workflows, and high-volume operator efficiency.
- Observability, logs, metrics, traces, alerts, support tools, health dashboards, incident workflow, status communication, runbooks, and post-incident review.
- Availability, performance, capacity, backup, restore, recovery point, recovery time, deployment, rollback, business continuity, and disaster-recovery evidence.
- Configuration guides, administrator and operator help, onboarding, release notes, support knowledge, pilot training, and customer-success instrumentation.
- Architecture must preserve a path for future role-based clinical-education modules, in-product guidance, and certification tracking.

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8. Products and operating profiles on the shared core

Product/profile	Scope role	Roadmap posture
Meridian Health Connect	Commercial clinic care-coordination operating system and shared national core.	Core commercial product
Ambulatory scheduling & documentation	Complete scheduling, check-in, encounter documentation, and portal capability.	Early production increment
Chronic care management	Care-plan, task, and monitoring-program depth for chronic-disease populations.	Primary clinical increment
Remote patient monitoring	Device onboarding, data intake, alerting, and adherence tracking.	Later transaction increment
Affiliated care-management profile	Multi-clinic oversight, program administration, and cross-clinic (contractually authorized) reporting.	Later shared-core profile
Behavioral health module	Screening, safety-plan workflow, and segregated documentation.	After core clinical foundation
Payer connectivity	Broader payer network coverage, prior-	Later; interleaved by

Product/profile	Scope role	Roadmap posture
expansion	authorization automation.	provider readiness
Specialty referral network	Curated specialist directories and enhanced closed-loop referral automation.	Later bounded capability
Clinical education modules	Role-based training, certification tracking, in-product guidance.	Late roadmap; architecture preserved now

9. Phased delivery sequence and Pilot Zero proof

Order	Increment	Controlled purpose
Architecture	Controlled architecture response	System/trust diagrams, component/data boundaries, ADRs, dependency plan, security/clinical-safety/integration/test designs, risks, assumptions, and decisions.
Pilot Zero	Foundation proof	Shared core, two isolated pilot environments, one validated scheduling-through-encounter lifecycle, one authorized basic care-plan proof, audit/security fixtures, controlled AI proof, and acceptance evidence.
1	Ambulatory scheduling & documentation	First complete production clinical-workflow increment.
2	Chronic care management	Care-plan depth, task automation, and monitoring-program readiness gating.
3	Remote patient monitoring	Device onboarding, data intake, alerting, and adherence tracking.
4	Payer eligibility & coverage	Real-time verification, authorization tracking, and reconciliation.
Interleaved	Behavioral health module	Inserted when shared dependencies and confidentiality controls are ready; not allowed to destabilize Pilot Zero.
Later	Affiliated care-management profile & referral network	After core clinical and payer foundations are stable.
Late roadmap	Clinical education modules & extended intelligence	Full learning centers, broader benchmarking, advanced predictive intelligence, and additional products.

9.1 Pilot Zero required capabilities

- Separate protected pilot environments with no patient, scheduling, encounter, or documentation data crossover.
- One validated scheduling-through-check-in-through-encounter path with complete history.
- One basic authorized care-plan creation and task-assignment proof.
- Tenant, entity, location, identity, role, permission, support-access, audit, workflow, configuration, document, and event foundations.
- Deterministic scheduling/eligibility-check kernel and reference fixtures; exact payer rules remain controlled and professionally gated.
- Provider simulations for payer eligibility, communications, and e-signature before live credentials are enabled.
- Read-only or recommendation-only AI proof using synthetic/approved test data; no autonomous consequential clinical action.
- Automated tests, negative/boundary/failure/replay tests, security evidence, backup/restore, rollback, pilot scripts, and Meridian-owner-approved acceptance package.

9.2 Pilot Zero exclusions

Pilot Zero is not the complete production product. Full chronic care management, remote patient monitoring, behavioral health, payer connectivity expansion, affiliated care-management oversight, advanced AI, benchmarking, clinical education modules, and national jurisdiction packs remain later increments unless expressly added through approved change control.

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10. Data, clinical safety, security, and AI controls

10.1 Canonical data and history

- Opaque internal identifiers; medical record number, national provider identifier, and other natural keys remain attributes with validation and provenance.
- Tenant, legal entity, location, patient, role, encounter, care plan, referral, monitoring reading, communication, document, rule, provider event, and audit records require explicit ownership and lifecycle.
- Effective date, occurrence/provider time, and recorded time must not be silently substituted for one another.
- Current state and lifetime history must reconcile; amendments, corrections, and referral outcomes remain visible.

10.2 Clinical safety and workflow integrity

- Every clinical-workflow event carries an approved purpose, source, effective date, actor, authorization, idempotency key, and correction relationship where applicable.

- Finalized clinical documentation is corrected by amendment or addendum; no destructive edit of clinical history.
- Threshold-based alerts and safety-plan triggers must be reliably delivered and require explicit acknowledgment; missed-acknowledgment escalation must be defined.
- Operational data must reconcile to source device/payer/lab confirmations where applicable.

10.3 Rules, overrides, and compliance

- Federal/state confidentiality requirements, consent rules, retention, and prohibited behavior must be effective-dated, sourced, reviewed, testable, and versioned.
- If no approved rule exists for a required jurisdiction or workflow, the software must block or remain test-only and alert authorized management.
- Overrides require explicit permission, reason, evidence, scope, original value, replacement value, approval, timestamp, and management reporting. Some controls must be non-overridable.
- Legal, clinical, security, privacy, and vendor authorities retain approval responsibility for their assigned subject matter.

10.4 Security, privacy, and AI

- Defense-in-depth tenant isolation, least privilege, multi-factor authentication, separation of duties, sensitive-field controls, encryption, secret management, monitoring, access reviews, incident response, backup/restore, and secure deployment.
- High-risk activity such as documentation amendment, behavioral-health record release, backdating, support access, bulk export, and safety-plan overrides requires heightened controls and exception reporting.
- AI outputs must identify model/provider, prompt/version, sources, data scope, confidence/limitations, human disposition, and audit trail.
- No AI autonomous authority for diagnosis, care-plan finalization, referral approval, prescription transmission, or other consequential clinical decisions.
- Cross-clinic benchmarks require legal/privacy review, anonymous cohorts, minimum population, suppression, and controls preventing identifiable clinic or patient disclosure.

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11. Integration and provider architecture

The vendor must recommend a replaceable adapter architecture. A provider integration is not complete merely because a request can be sent. Every integration needs credential ownership, consent/data rights, sandbox/live separation, timeout/retry, duplicate handling, status inquiry, webhook validation, reconciliation, monitoring, support ownership, data export, and exit treatment.

Integration class	Expected capability

Integration class	Expected capability
Payer eligibility & authorization	Real-time/batch eligibility checks, coverage summary, prior-authorization status, and reconciliation.
Laboratory & diagnostic results	Order transmission where applicable, result intake, structured/unstructured result handling, and provenance.
Remote monitoring devices	Device data intake, status/health signals, and vendor-specific protocol adapters.
Communications	Email, SMS, voice/telephone, secure messaging, templates, consent, quiet hours, delivery, and opt-out.
Documents/e-sign	Template generation, consent forms, signatures, identity, evidence package, completion status, retention, and export.
Identity verification	Patient identity proofing for portal access and sensitive-record requests.
Directory/referral network	Specialist and facility directory data, referral transmission, and status updates.
AI/model providers	Approved model gateway, data-use terms, logging, evaluations, cost, fallback, monitoring, and provider portability.

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12. User experiences, portals, mobile, and training

Experience	Primary workflow coverage
Clinic owner/medical director	Cross-location performance, exceptions, approvals, quality/operational intelligence, audit, and controlled benchmark views.
Location manager	Daily scheduling, staffing, task queues, exceptions, and approvals.
Front-desk/scheduling staff	Scheduling, eligibility check-in, waitlist, reminders, and portal support.
Clinician	Documentation, care plans, referrals, monitoring review, and messaging.
Care coordinator	Referral tracking, task management, monitoring-program adherence, and patient outreach.
Behavioral health staff	Screening workflow, safety-plan documentation, and segregated case management.
Affiliated organization oversight	Contractually authorized cross-clinic performance, support, and exception views without uncontrolled tenant access.
Patient	Portal/app for scheduling, messaging, forms, care-plan visibility, and document access as enabled.
Meridian support/admin	Tenant onboarding, configuration, consented support, health/incident, integration, deployment, and audit tools with privileged controls.

12.1 Training posture

Each delivered phase requires operator instructions, administrator configuration guidance, support runbooks, release notes, known limitations, and pilot training. Full clinical-education modules are later scope, but the selected architecture must allow future role-based manuals, teaching videos, assessments, certification, version-aware learning, and in-product guidance.

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13. Nonfunctional and operational requirements

Quality area	Response expectation
Scalability	Support multi-tenant growth, separately scalable workloads, large document/media volumes, telemetry, background jobs, reporting, and integration traffic.
Availability	Vendor proposes measurable service levels, maintenance windows, degraded modes, dependency isolation, and customer communication.
Performance	Vendor proposes user-response, API, batch, report, and search targets plus load-test evidence.
Security	Secure SDLC, code/dependency scanning, threat modeling, penetration testing, tenant-isolation testing, secret/key controls, and incident evidence.
Data protection	Encryption, backups, point-in-time recovery where needed, restore tests, retention, legal hold, deletion treatment, and disaster recovery.
Observability	Logs, metrics, traces, audit, health, synthetic checks, integration status, alerts, dashboards, on-call/escalation, and post-incident review.
Accessibility/usability	Responsive, keyboard-accessible, readable, plain-language, efficient for repetitive clinical workflows, and tested with representative users.
Portability	Documented export formats, migration utilities, API access, provider replacement, configuration portability, and termination assistance.
Localization/time	Location time zones, effective dates, calendar rules, daylight-saving treatment, formatting, and future language support.
Maintainability	Modular ownership, clear APIs/events, ADRs, automated tests, CI/CD, infrastructure as code, versioned migrations, feature flags, and rollback.

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14. Engineering delivery, ownership, and acceptance

14.1 Required working controls

- Meridian-approved repositories, cloud accounts, domains, environments, CI/CD, secrets, storage, monitoring, and third-party accounts; no vendor lockout or sole possession of production access.
- Named lead architect, project manager, security responsibility, QA responsibility, and stable development team with disclosed subcontractors and locations.
- Controlled backlog, permanent requirement/decision/risk/question IDs, architecture decision records, change requests, version history, and release evidence.
- No silent assumptions. Every gap, conflict, dependency, risk, exclusion, or recommendation receives an owner, impact, due date, status, and disposition.
- Two to three structured progress touchpoints per week during active delivery, plus demonstrations, written status, risks, decisions required, and updated forecast.
- No production data in uncontrolled environments; synthetic or approved masked data for development and test.
- No live provider credentials, production patient data, patient communications, or regulated production rules until their specific approvals and readiness gates are complete.

14.2 Required technical deliverables

- System context, trust-boundary, logical component/service, deployment, data, event, integration, security, and environment diagrams.
- Canonical data model, ownership/lifecycle definitions, API/event contracts, workflow/state models, permission matrix, rule/configuration model, and error/failure contracts.
- Source code, infrastructure as code, database migrations, automated tests, fixtures, CI/CD, build/deploy instructions, configuration, and rollback procedures.
- Threat model, security/privacy design, tenant-isolation evidence, incident plan, backup/restore evidence, dependency inventory, and vulnerability results.
- Administrator, operator, support, integration, API, data dictionary, runbook, release note, training, and known-limitation documentation.
- Acceptance matrix tracing requirements to tests, demonstrations, evidence, defects, disposition, and the Meridian owner's final acceptance decision.

14.3 Ownership and commercial protection

- Meridian must own or possess perpetual transferable rights to all custom source code, schemas, documentation, configuration, tests, designs, prompts, workflows, and deliverables, subject only to disclosed pre-existing/open-source components.
- Every third-party or open-source component must be identified with license, purpose, version, security/support posture, and replacement path.
- The eventual contract must address confidentiality, intellectual property, data processing, security, breach/incident duties, personnel/subcontractors, warranty,

support, transition assistance, termination, and delivery of current working materials.
- Milestones must be tied to objectively reviewable deliverables and accepted evidence, not calendar passage or unsupported percent-complete claims.

15. Required proposal response

Response rule: Do not return a one-line summary. A proposal that omits phase breakdowns, named personnel, assumptions, exclusions, dependencies, acceptance terms, or a description of ongoing support will be treated as incomplete. This attachment requests scope, architecture, staffing, and delivery-approach information only; do not include pricing, rate cards, or budget figures in the response to this attachment. Commercial terms will be addressed separately with shortlisted firms.

Response section	Required content
Executive response	Understanding of the business, proposed engagement, major risks, and why the team is qualified.
Architecture recommendation	Recommended stack, service boundaries, data architecture, deployment, security, integrations, AI gateway, and major alternatives/tradeoffs.
Phase plan	Dependencies, milestones, durations, staffing, entry/exit criteria, demonstrations, deliverables, and acceptance evidence for every phase.
People	Actual lead architect, project manager, developers, QA, DevOps/cloud, security, UX, mobile, clinical-informatics advisor, locations, time zones, availability, and replacement policy.
Assumptions	Every fact assumed true in producing the schedule; identify the impact on schedule if false.
Exclusions	Every item not included, deferred, dependent on Meridian, or dependent on an outside provider.
Third parties	Expected providers, licenses, device/hardware dependencies, infrastructure, communications, and AI dependencies.
Quality/security	Test strategy, tenant isolation, secure SDLC, performance, accessibility, backup/restore, rollback, warranty, and defect handling.
Ownership/support	Repository/account ownership, IP, source delivery, documentation, deployment, warranty, maintenance, SLA/on-call options, transition, and termination assistance.
Response validity	Proposed start date, earliest responsible completion, and conditions for proceeding.

16. Bidder qualifications and evaluation

16.1 Minimum qualifications

- Demonstrated delivery of multi-tenant SaaS with strong tenant isolation, roles/permissions, audit, and controlled production operations.
- Experience with regulated health-information systems, clinical workflows, or comparably sensitive systems.
- Capability across architecture, backend, database, web, mobile, integrations, DevOps/cloud, security, QA automation, documentation, and support.
- Ability to explain architecture and clinical-workflow tradeoffs clearly to a nontechnical business owner without hiding behind jargon or generic AI output.
- Willingness to work through controlled requirements, evidence, pilot gates, Meridian-owned repositories/accounts, confidentiality, and IP terms.
- Healthcare, payer-integration, remote-monitoring, or behavioral-health experience is strongly preferred but does not replace core engineering proof.

16.2 Evaluation weighting

Evaluation category	Weight
Architecture and regulated-workflow understanding	25%
Tenant isolation, security, privacy, audit, and recovery	20%
Comparable delivered systems and named team	15%
Phased plan, testing, pilot, and acceptance evidence	15%
Communication, assumptions, and transparency	15%
Scalability, maintainability, support, and transition	10%

Appendix A. Required response-structure checklist

Item	Required content
A	Architecture response and final Pilot Zero plan
P0	Pilot Zero foundation proof
1	Ambulatory scheduling & documentation
2	Chronic care management
3	Remote patient monitoring
4	Payer eligibility & coverage
I-1	Behavioral health module
L-1	Affiliated care-management profile
L-2	Specialty referral network
L-3	Clinical education modules & extended intelligence
ALL	Complete program planning narrative + confidence level

For each row, attach duration, team, milestones, dependencies, assumptions, exclusions, and acceptance deliverables.

Appendix B. Mandatory bidder questions

1. Identify the individual lead architect and project manager who will actually perform the work. What percentage of their time is committed?
2. Provide three comparable systems, describing tenant isolation, regulated-data handling, scale, your exact role, current status, and a verifiable reference or demonstration.
3. How would you prevent data leakage between two independently protected clinic environments while allowing one authorized patient-transfer workflow?
4. How would you design immutable clinical documentation history, amendment/addendum workflow, and audit reconciliation?
5. How would you implement effective-dated federal/state confidentiality packs and clinic setup choices without hard-coding every rule into workflow code?
6. Which architecture and technology stack do you recommend, what alternatives did you consider, and what are the major tradeoffs and vendor-lock-in risks?
7. How will you protect Meridian’s ownership and continuous access to repositories, cloud accounts, secrets, documentation, deployments, and working deliverables?
8. Which outside providers are necessary, which can be simulated initially, and how does the system behave during provider failure or unknown results?
9. Which AI capabilities would you permit automatically, which require human approval, and how will models, prompts, sources, evaluations, and incidents be governed?
10. Can architecture plus Pilot Zero be responsibly completed within five months or sooner after written authorization? State the assumptions, critical path, team, risks, and anything that must move later.
11. What parts of the complete program cannot yet be responsibly scoped, and exactly what discovery or requirements are needed before you will commit to a schedule?
12. Provide your warranty, support, maintenance, SLA/on-call, security-response, knowledge-transfer, transition, and termination-assistance approach.

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